

POLICYREADY • TEXAS

How to Open a **Host Home Business** in Texas

Texas's HCS Host Home / Companion Care model for people with IDD

Step by Step

*From “where do I start?” to your first payment —
whether you go through a program provider, or build your own.*

A complete, plain-language guide for real people — not lawyers or consultants.
Figures current for 2026 • Verify all details with the State of Texas.

Read this first

This is a plain-language roadmap built from Texas's actual rules, agencies, and bill-code crosswalk. It is not legal, tax, or billing advice, and the details change — rates, codes, and regulations update regularly. Before you act, confirm the current specifics with HHSC, your LIDDA or program provider, an attorney, an accountant, and an insurance broker. Every rule, code, and citation here is real and pulled from the state's own sources — but always check the current version before you rely on it.

What this guide covers

- Part 0 — The big picture: what a host home is in Texas, who the players are, and the two ways in.
- Part 1 — Path A: providing host home / companion care through an HCS program provider (the fastest way).
- Part 2 — Path B: becoming your own HCS program provider and billing Medicaid yourself.
- Part 3 — Who you need on your team.
- Part 4 — The home: what your house actually needs (sprinklers, escapes, the safety checklist).
- Part 5 — Getting your first person placed.
- Part 6 — How you get paid: the real billing codes, the LON system, and your first payment.
- Part 7 — What it costs and how long it takes.
- Part 8 — When they say “check locally”: who to call and exactly what to ask (Houston & Dallas worked out).

PART 0

The Big Picture

Opening a host home in Texas is not like opening a normal small business, and the reason people get lost is that the information lives in a dozen different places. Here it is in one place.

What a host home actually is in Texas

In Texas the host-home model is called Host Home / Companion Care (it used to be called foster care). It's a private home — your home — where a person with an intellectual or developmental disability (IDD) lives with you and receives support, as a home-like alternative to a group home. It's a service under the HCS (Home and Community-based Services) Medicaid waiver, run by the Texas Health and Human Services Commission (HHSC). HCS group homes are capped at four beds; host home / companion care is the one-person-in-a-family-home option.

The players — learn these names, you'll deal with all of them

- HHSC — the Texas Health and Human Services Commission. It runs the HCS waiver, contracts the providers, sets the rates, and certifies/inspects settings (through its Long-Term Care Regulatory division, LTCR).
- HCS waiver — the Medicaid program that funds residential and other services for people with IDD.
- LIDDA — your Local Intellectual and Developmental Disability Authority: the county-level “front door” that determines eligibility, manages the interest list, and assigns a service coordinator.
- Service coordinator — the LIDDA staffer who writes the person's Person-Directed Plan (PDP) and Individual Plan of Care (IPC).
- Program provider — the agency that holds the HCS contract and delivers (and bills for) services. On Path A, you contract with one; on Path B, you become one.
- TMHP — Texas Medicaid & Healthcare Partnership, the state's claims system where HCS claims are submitted.

The fork in the road — there are two ways in

This decides everything, so understand it before you spend a dollar.

- Path A — Provide host home / companion care under an existing HCS program provider. You partner with an agency that already holds an HCS contract. They recruit and place the person, handle the Medicaid billing, and support you; you provide the home and the day-to-day care, usually as a contractor. Fastest, lowest-risk — this is how almost everyone should start.
- Path B — Become your own HCS program provider. You apply to HHSC through its open-enrollment contracting process, pass a readiness review and site survey, enroll with Medicaid, take referrals from LIDDAs, and bill HHSC yourself. A much bigger lift (and a rigorous application) — but it's the real business, with the real revenue and control.

PART 1 • PATH A

Provide Host Home / Companion Care

Start here. You can always grow into your own program later.

Step 1 — Make sure it fits your life

A host home means an adult with an IDD lives in your home, with you, and you support them through everyday life — meals, routines, appointments, the good days and the hard days. Your home becomes their home. Everyone in your household who's 18 or older will be part of the background check, so this is a family decision. Sit with it honestly; if you've got the space, the time, and the heart for it, you're ready for step two.

Step 2 — Choose an HCS program provider

On this path you don't contract with the state yourself — you partner with an agency that already holds an HCS contract. Think of them as your teammate: they find the person you'll support, handle the Medicaid billing, train and monitor you, and pay you (usually as a contractor). Call your LIDDA for a list of HCS program providers in your area, and interview more than one. Ask how they support host-home providers, how often they place people, and how and when they pay.

Step 3 — Pass the background checks

Before anyone moves in, you and the adults in your home are screened — criminal history plus checks of the Employee Misconduct Registry and Nurse Aide Registry, and you can't be on the HHS Office of Inspector General exclusion list. This is non-negotiable; everything here comes back to protecting a vulnerable person. Your program provider runs the screening and walks you through it.

Step 4 — Get your home ready and pass inspection

Your home has to meet the HCS residential requirements (TAC §565.23). See Part 4 for the full checklist. For host home / companion care, the program provider inspects your home before move-in and at least quarterly after — and HHSC/LTCR can inspect at any time.

Step 5 — Complete your training

You'll complete the training your program provider and the HCS rules require — typically abuse/neglect/exploitation prevention and reporting, person-centered service, CPR/First Aid, and any medication or specialized training the person's plan calls for. It's there so that the day something goes sideways, you already know what to do.

Step 6 — Get matched and welcome your resident

The LIDDA's service coordinator and your program provider match you with a person whose needs fit your home, and there's a pre-placement visit and enrollment meeting. You'll sign a written residential agreement (covering room and board) with the person, and your provider agreement. Take the match seriously — a good fit is what makes this last.

Step 7 — Put your policies in writing

Your policies and procedures — medications, emergencies, confidentiality, incident reporting, and the rights of the person you serve — must be documented. This is exactly what PolicyReady's Texas host home manual is built for: ready to customize, just add your details.

Step 8 — Live it, document it, get paid

Follow the person's plan and keep good records. Your program provider monitors your home quarterly and supports you. On this path, the provider bills HHSC through TMHP and pays you per your contract for the days you support the person. That first payment is the moment it all clicks.

PART 2 • PATH B

Become Your Own HCS Program Provider

More work, more money, more control — and a rigorous application. Here's the whole thing, in order.

Step 1 — Set up the business

- Form your entity (usually an LLC) with the Texas Secretary of State.
- Get your EIN/FEIN from the IRS (free) — you'll submit the IRS CP-575 or 147c letter with your application.
- Get a Type 2 (organizational) NPI through NPPES (free) — HHSC requires the NPPES verification with your application.
- Open a business bank account and set up bookkeeping.

Step 2 — Meet the program-manager requirement

HHSC requires your program manager to have at least three years of paid experience planning and providing HCS-type services to a person with an intellectual disability or related condition (verified in writing), and to pass the provider competency examination with a score of at least 90 percent. This is one of the biggest gates on Path B — line it up early.

Step 3 — Apply through HHSC open enrollment (the core)

HHSC contracts for HCS on a non-competitive, open-enrollment basis — anyone who meets the qualifications can be awarded a provisional contract. You apply online through the HHSC CAMP portal and submit the required forms (for example Form 2031 Governing Authority Resolution), your NPI and EIN verifications, the program manager's resume and competency exam, and more. Rules for the provisional-contract process are in TAC §52.31.

Step 4 — Pass the readiness review & site survey

HHSC's LTCR division conducts a readiness review and, for licensed settings, an on-site survey. HHSC may issue a temporary license for up to 180 days while the survey is completed. You'll submit floor plans, fire-safety documentation, a staffing plan, and emergency procedures for each home.

Step 5 — Enroll with Texas Medicaid & set up billing

Complete Medicaid provider enrollment (TAC Title 1, Chapter 352) and set up to submit claims through TMHP TexMedConnect or EDI. Be ready for Electronic Visit Verification (EVV) on the services that require it.

Step 6 — Build your operation

Put your policies and procedures in place (PolicyReady manuals), stand up your service delivery and QA, and recruit, screen, and inspect your host homes — the same standards described on Path A. Subscribe to HHSC's GovDelivery updates (required by contract).

Step 7 — Take members, deliver services, and bill

Accept referrals from LIDDAs (the person must be HCS-enrolled), place them, deliver services, and bill HHSC through TMHP. Questions on contracting: HHSC IDD Waiver Contract staff, 512-438-3234, or HCSPolicy@hhs.texas.gov. This is the finish line — covered in Parts 5 and 6.

PART 3

Who You Actually Need (Staffing)

On Path A (you're the host-home provider): you don't build a staff. At most you line up screened, trained substitute/relief caregivers so the person is never left without support when you're sick or away. The program provider supplies the oversight, nursing, service coordination support, and billing.

On Path B (you're the program provider), plan for these roles — one person can wear several hats at the start:

- Program manager — the 3-years-experience, 90%-exam role above; owns program compliance.
- Administrator / owner — runs the business and the HHSC contract.
- Registered Nurse — for health oversight, medication, and nursing services; often part-time or contracted at first.
- Service delivery & QA staff — recruit, train, and monitor host homes; complete the quarterly inspections and documentation.
- Billing / administrative support — submits TMHP claims (and EVV where required) and tracks authorizations and payments; many small agencies outsource this at first.
- Host-home providers — your contracted caregivers, each screened, trained, and in an inspected home.

Staff qualification rules are in the HCS Billing Requirements and Appendix C of the HCS waiver — confirm your specifics with HHSC.

PART 4

The Home: What Your House Actually Needs

The one thing to understand first

A host home is your private residence with one person living in it — not a facility. The heavy institutional “Life Safety Code” survey (HHSC Forms 5604/5606) is aimed at four-person homes; a host home / companion care setting is held to the residential requirements in TAC §565.23 and is monitored by your program provider, with HHSC able to inspect at any time. That's why you can open a host home in an ordinary house.

“Do I need fire sprinklers?” — the question everyone asks

Here's the exact Texas rule, and it's clearer than most states. Each bedroom must have TWO means of escape — unless the home has a maintained fire sprinkler system, in which case one is allowed. In other words, sprinklers are not required; they're an alternative that lets you have a single escape from a bedroom. A normal host home simply provides two ways out of each bedroom (for example, a door plus an escape window) and skips sprinklers entirely.

THE SAFETY CHECKLIST (TAC §565.23)

- Two means of escape from each bedroom (or one, if the home has a maintained fire sprinkler system).
- Working smoke alarms in each bedroom and immediately outside each bedroom, on every floor including basements.
- Fire extinguisher(s) available and maintained.
- A written emergency evacuation plan, practiced with the person.
- Bedroom doors that can be locked but are operable — openable from inside without a key and by staff in an emergency.
- Hot water at a safe temperature to prevent scalding, based on a face-to-face assessment of the person's abilities.
- No conditions hazardous to health or safety; hazardous materials stored safely.
- Working utilities, safe electrical, clean and sanitary conditions, and accessible exits kept clear.

INSPECTIONS

- For host home / companion care, the program provider inspects the home before move-in and at least quarterly (TAC §565.25).
- HHSC / LTCR may enter and inspect at any time.
- If you ever move to a four-person residence, the heavier Life Safety Code survey (Forms 5604/5606) applies — another reason many people stay with the single-person host home model.

Two more housing things that trip people up

- Zoning — “can I even do this in my neighborhood?” Homes for people with disabilities are protected as a residential use under the federal Fair Housing Act, so a host home is generally allowed in residential areas like any family home. Texas cities vary a lot on local rules (see Part 8 — Houston famously has no zoning at all), so confirm with your city or county.
- Room and board vs. the Medicaid payment. In Texas, the person pays their own room and board (usually from SSI) under a written residential agreement; Medicaid pays for the service, not the rent and food. If room and board goes unpaid, HHSC can even deny the residential service — so keep that agreement current.

PART 5

Getting Your First Person Placed

You can't get paid until someone is living with you. Here's how that happens.

1. The person qualifies and has a slot. HCS has a long interest list (well over 100,000 Texans), so the person you serve has typically waited years for a funded HCS slot. Eligibility and the interest list are handled by the LIDDA.
2. The LIDDA builds the plan. The service coordinator develops the person's Person-Directed Plan (PDP) and Individual Plan of Care (IPC), which spell out the services authorized.
3. The LON is assigned. HHSC assigns a Level of Need (LON) from the person's ICAP assessment score and ID/RC Assessment — and that LON drives your payment (see Part 6).
4. The match. The LIDDA and your program provider match a person whose needs and personality fit your home, with a pre-placement visit and enrollment meeting.
5. Move-in and sign. The person signs a written residential agreement (room and board) and moves in. Now you're delivering a billable service.

PART 6

How You Get Paid: Codes, the LON System & Your First Payment

How Texas decides your rate: the LON

Host home / companion care is reimbursed based on the person's Level of Need (LON) — an assignment HHSC derives from the Inventory for Client and Agency Planning (ICAP) service level score plus selected items on the ID/RC Assessment. Higher need = a higher LON = a higher rate. The LON levels used for HCS residential are 1, 5, 6, 8, and 9.

The actual billing codes

For claims, the service maps to a national HCPCS code plus a Texas “bill code” that pins down the exact setting and LON. For a host home, the HCPCS code is H2016, and the Texas bill code changes with the LON:

Setting	HCPCS	Texas bill code by Level of Need (LON)
Host Home / Companion Care	H2016	M0122 (LON 1) · M0123 (LON 5) · M0124 (LON 6) · M0125 (LON 8) · M0126 (LON 9)
Supervised Living (H2016 + modifier UH)	H2016 UH	M0166 – M0170 (LON 1/5/6/8/9)
Residential Support (H2016 + modifier UJ)	H2016 UJ	M0149 – M0153 (LON 1/5/6/8/9)

So the everyday code for a host home is **H2016, billed under Texas bill code M0122–M0126 depending on the person's LON**. Claims go through TMHP (TexMedConnect or EDI), and some HCS services require Electronic Visit Verification (EVV). On Path A your program provider files these for you; on Path B you file them yourself.

How much — and why it changes

- Who sets it: rates are set by HHSC's Provider Finance Department under the HCS rate methodology in TAC Title 1, Chapter 355, and published on the HHSC Provider Finance rate sheets (typically effective September 1 each year).
- Why it moves: the rates are tied to the state budget the Texas Legislature sets every two years and to attendant-wage assumptions built into the rate model — so figures change, and there isn't one number to memorize.
- The takeaway: find the current dollar figure for the LON you're serving on the HCS rate sheet (Provider Finance Department), or ask your program provider, before you build a budget.

Your first payment

6. Confirm the authorization. The person's IPC and LON set what can be billed. Make sure it matches what you're providing.
7. Deliver and document. Keep your service records (and complete EVV where required).
8. The claim goes to TMHP — H2016 with the right M-code for the LON. On Path A your provider submits it and pays you per your contract; on Path B you submit it yourself.
9. Get paid — and never balance-bill. Medicaid pays the service; the person owes only their agreed room and board. Never bill the person or family for a covered service.

PART 7

What It Costs & How Long It Takes

Timeline

Path A (providing under a program provider) can move relatively quickly — screening, training, and a home inspection — often a matter of weeks to a couple of months once you've chosen a provider and a match is available. Path B (becoming your own HCS program provider) is a months-long process: the open-enrollment application, the program-manager competency exam, the readiness review, and the site survey (with a temporary license possible for up to 180 days). Confirm current timelines with HHSC.

Startup line items

Path A is inexpensive — mostly your time, background-check fees, training, and getting your home ready (smoke alarms, extinguisher, safe exits, minor fixes). Path B adds business formation, an NPI (free), the HCS application, insurance, your policies and procedures, and the infrastructure to deliver and bill for services. Confirm current fees with HHSC.

PART 8

When They Say “Check Locally”: Who to Call & What to Ask

Some things are handled by the State (HHSC), some by your LIDDA or program provider, and some by your local city or county (zoning and building). Here's who to call and exactly what to ask, so you get a straight answer instead of getting bounced around.

The statewide offices (same for everyone)

For...	Contact
Becoming your own HCS provider (contracting)	HHSC IDD Waiver Contract staff — 512-438-3234 · HCSPolicy@hhs.texas.gov
HCS program & certification info	hhs.texas.gov → Long-Term Care Providers → HCS
Billing codes (bill-code crosswalk) & claims	TMHP — tmhp.com → HCS/TxHML Bill Code Crosswalk
Rates (HCS rate sheets)	HHSC Provider Finance Department — pfd.hhs.texas.gov
Find your LIDDA / get on the interest list	hhs.texas.gov → Local IDD Authority (LIDDA); or call 1-855-937-2372

The local calls — and what to ask on each one

① YOUR LIDDA — YOUR MOST IMPORTANT LOCAL CALL

- “Which HCS program providers in this area place people in host homes / companion care, and can you connect me with them?”
- “How does someone get matched to a host home here, and what do you look for in a host-home provider?”
- “Can you confirm my county's LIDDA and how referrals work in this area?”

② CITY OR COUNTY PLANNING / ZONING — “CAN I LEGALLY RUN A HOST HOME HERE?”

- “I want to open a host home — a residence where one adult with a developmental disability lives with me and receives support. Is that an allowed residential use at [address]?”
- “Are there any local rules, permits, or occupancy limits I should know about? Can you confirm in writing?”
- If they hesitate: homes for people with disabilities are protected as a residential use under the federal Fair Housing Act — ask them to point to the specific rule.

③ BUILDING / FIRE — ONLY IF YOU'RE REMODELING, OR AS REQUIRED LOCALLY

- “I have an existing home I'll use as a host home for one person. If I'm not changing the structure, do I need any permits?”
- “Any local fire requirements for a single-resident host home (alarms, extinguisher, exits)?”

Fully worked examples — Texas's two biggest metros

Most people live in or near one of these two, so each one's real call list is done for you below. If you're elsewhere, use these as a template.

HOUSTON (HARRIS COUNTY)

- LIDDA: The Harris Center for Mental Health and IDD — 713-970-7000 (Harris County's IDD front door).

- Zoning: here's Houston's famous quirk — the City of Houston has NO zoning ordinance. There are still development rules (lot size, setbacks, deed restrictions), handled by the City Planning & Development Department; ask the Development Services Planner of the Day at 832-393-6624.
- Building permits: City of Houston Permit Center, 1002 Washington Ave. If your home is in unincorporated Harris County, permits go through the Harris County Office of the County Engineer, and fire through the Harris County Fire Marshal.
- The local quirk here: no zoning inside the city — but deed restrictions can still limit use, so check your subdivision's deed restrictions too.

DALLAS (DALLAS COUNTY)

- LIDDA: Metrocare Services — Dallas County's IDD front door (eligibility, interest list, service coordination).
- Zoning & building: City of Dallas Development Services Department — handles zoning, permits, and inspections (Dallas does have zoning — the opposite of Houston).
- If your address is in a suburb or unincorporated Dallas County, contact that city's development services (or Dallas County) instead.
- The local quirk here: unlike Houston, Dallas has a full zoning code — confirm your property's zoning district allows a residential/host-home use.

How to find your own city or county's numbers

10. Start with your LIDDA and your program provider — they know the local landscape.
11. For zoning, search “[your city] TX planning and development” or “[your county] TX development services,” or call your city/county main line and ask for Planning/Zoning.
12. Not sure who covers you? Call 2-1-1 Texas and ask.

CORE RULES & REFERENCES

- TAC Title 26, Chapters 263 and 565 — HCS Program and Community First Choice (certification standards); §565.23 residential requirements, §565.25 inspections, §565.7 staff.
- TAC Title 26, Chapter 52 — Contracting for Community Services (Path B).
- TAC Title 1, Chapter 352 — Medicaid provider enrollment; Chapter 355 — HCS reimbursement/rate methodology.
- HCS & TxHmL Bill Code Crosswalk (TMHP) — the HCPCS + Texas bill codes (H2016; M0122–M0126 for host home by LON).

You can do this.

Opening a host home is real work — but it's a real, walkable path, and people across Texas do it every day for the more than 100,000 Texans waiting for a place to call home. You don't have to be a lawyer or hire a consultant. You need a safe home, a good heart, and a clear map — and now you have the map. Whether you start the fast way through a program provider or build your own, this guide takes you from your very first question all the way to your first payment.

Take it one step at a time, verify the current details with the State as you go, and lean on your LIDDA and your program provider — that's what they're there for.

Prepared for PolicyReady.org. Grounded in Texas HHSC rules, the Texas Administrative Code, and the HCS & TxHmL Bill Code Crosswalk as of 2026. This guide is general information, not legal, tax, or billing advice. Requirements, rates, and codes change — always verify current details with the State of Texas before you act.